

STAGE 1 OF 5 · CYTOKIND 2.0

Discovery

Everything known about CytoKind, December 2025 → August 2026. Fourteen sources, eight months. Every fact tagged verified, unverified or unknown. Nothing estimated, all money on the \$2,000 floor unit.

The master audit. Everything known about CytoKind, consolidated from every call, every system, every document and every live artifact, Dec 2025 → Aug 2026. This is stage 1 of 5: AUDIT → PROBLEMS → SOLUTIONS → PRIORITIZATION MATRIX → ROADMAP.

Scope of the engagement: CLIENT ACQUISITION · FUNNEL · SALES. Nothing else. Product, clinical, regulatory and fundraising appear here only as context or constraint — they are not workstreams.

Confidence: ✓ verified from a primary source · ! unverified/second-hand · ? unknown, must be asked.

Updated 16 August 2026. Where a figure changed, the 14 Aug value is the one shown. Fourteen sources, eight months. Supersedes every earlier version of this file.

o. LIVE STATE — 14 August 2026 ★ this table wins over anything below it

Source: Adrian call 14 Aug (08-CALL-adrian-2026-08-14.md) + Meta ad-library measurement the same day.

Devices sold, lifetime	6	"and these are all insurance sales" — all six derm, insurance-covered, patient paid \$0 ✓
CRM contacts	334	was 262 on 4 Aug ✓
Close rate	~1.8%	6 of 334. Adrian: "most of those are MS. The close rate out of DERM is pretty good" ✓
New contacts/day	~20	the highest volume in company history ✓
Prescriptions parked	18–20	was 14. Patients who already decided; nobody chases the signature ✓
JotForms completed, ever	4	"If you fill out that form, it's a sale, basically" ✓
Active paid ads	4	2 creatives × A/B, live since 11 Aug, behind 2 landing pages ✓
Ad targeting	half the spend still points at MS	0 devices ever sold there ✓
Phone connect	1 in 15–20	one part-time person ✓
Email connect	1 in 40	deliverability never verified !
Drip campaign	25% open · 1% CTR	4 emails. "It's not leading to anything" ✓
Monthly shortfall	\$30,000	20-device target vs ~5 actual, at the \$2,000 floor unit ✓
Business metrics that exist	1 of 12	eleven are not measured ✓

★ THE BREAK POINT, in the words of the person on the phones:

"They fill out the form, which is a landing page, and then they hit submit... and that's the end of the experience." · "So we're letting go of them while we have them."

They are returned to Facebook. No thank-you page, no education, no routing, no next step, no cost statement. **Some buyers reach the phone still not knowing insurance covers the device.** One said "it sounds like a scam", went and checked the site himself, came back a day later and bought. **He self-rescued. Nothing in the funnel does that for anyone else.**

0.1 The one-paragraph read

CytoKind is a two-person US distributor of Phothera NB-UVB phototherapy devices with a genuinely world-class earned-media asset (a Scientific American cover), a defensible scientific position in MS/autoimmune, and no working commercial engine underneath any of it. The ads work. The landing page works. Roughly twenty people a day now arrive — the most ever. Then the funnel stops at the submit button, and everything between there and the sale is one part-time person on a phone connecting once in twenty. It spends half its ad budget on the one segment that has never produced a sale, hides its best-converting asset behind its worst-performing channel, captures leads before it has educated them, holds 18–20 signed-off patients waiting on a signature nobody chases, and has never invoiced roughly fifty devices it has already delivered. Every individual link in the chain has an owner. **The chain has none.** That is the vacancy, and it is the engagement.

0.2 Method — how this audit was built

- Rule 01 — Artifact-anchored, not memory-anchored.** Never estimate a count. Ask for the specific recent example and open the system on the call. Every number here came out of a system while someone watched.
- Rule 02 — Nothing is estimated.** Where a metric does not exist, this document says so and names who holds it. The absence is itself the finding: **eleven of twelve business metrics are unmeasured.**
- Rule 03 — Priced on the floor unit.** All money is computed on the **\$2,000 Phothera 200**, the cheapest device. Every figure is therefore a minimum, and no answer is needed from the client on margin or ASP. **Margin per unit and ASP were deliberately never asked — do not raise them again.**
- The five stages:** 01 Audit (current state, this document) → 02 Problems (ranked on money moved) → 03 Solutions (one fix per problem) → 04 Prioritization (impact vs effort) → 05 Roadmap (sequenced execution, the only client-facing document of the five).

1. Company

Legal	Cytokind, Inc. — 18 Forest Ring Drive, Intervale, NH 03845, USA ✓
Site	cytokind.net (WordPress) ✓
What they are	US distributor of Phothera NB-UVB phototherapy devices. They do not manufacture ✓
Market	DTC (patients) + a clinical line for derm practices. US direct; rest of world routed to international distributors ✓
Evolv relationship	Client since ~mid-2025, currently ~€400/mo ⚠️. Adrian is embedded inside the company
Capital	Founders in ~\$500K · need ~\$4M · a family foundation committed \$500K but only after another \$1M ⚠️ · Boston Scientific has invested and holds an option to buy ✓ (J, 2026-08-06)

THE SEATS — AND THE DISTANCE BETWEEN THEM

SEAT	HOLDER	COVERS	STAFFING
Company and product	John MacMahon, CEO	Science, manufacturer relationship, investor story, every delegated decision	Full-time, unpaid, self-funded
Sales and CRM	Adrian Delli Colli	Outbound calls, inbound replies, CRM, prescription hand-off, insurance conversations	Part-time, one person

SEAT	HOLDER	COVERS	STAFFING
Creative	Craig	Ad video creative, personas, the sales script	Part-time, unpaid
Paid media	983 Group (Tom, Nigel)	Meta campaigns, targeting, budget, captions, the landing page	Retained — the only party being paid
Impression → click → form → call → prescription → JotForm → shipment → invoice	—	—	UNOWNED

PEOPLE

- **John MacMahon — CEO.** Two decades in medical device innovation out of **Stanford Biodesign**; six companies napkin-to-exit, eight total ✓. jmacmahon@cytokind.net · +1 603-264-7751 ✓
- **Stated goal: get out of the operator seat and focus on fundraising** ✓ — the emotional driver of the whole engagement.
- DTC is explicitly new to him: "we have more of a direct-to-consumer voice, which is new to me." ✓
- Takes **no salary**, funds the company personally !
- **Adrian Delli Colli — Evolv side, embedded internally.** The entire sales function, part-time, \$2,500/mo !. Has personally contacted **200+ people** ✓. **Now based in Campania, Italy** (no longer San Diego) — same timezone as Whiz ✓. Speaks to John **2–5x per week** ✓ — anything said to Adrian reaches John.
- **Craig** — content/creative. ! **Correction to the earlier record: Craig makes the video creative, not Tom.** The psoriasis ad is his AI-generated video; the MS ad is a podcast clip ✓. Also runs Claude internally on Google Ads, the website, landing pages and dwell time ✓. Wrote the sales script ! — and **has never made a sales call** !. Unpaid !
- **Tom — 983 Group.** Writes the ad captions, sets the ads up, **vibe-coded the landing page**, owns the ActiveCampaign build ✓. Also runs **Bioness**, another medical device account ✓
- **Nigel — 983 Group.** Owns the contract and the PR/earned-media side, which genuinely works ✓
- **Alexander Dugom** — investor contact introduced by Whiz. Investment **gated on sales proof-of-concept** ✓
- **Richard Weller** — advisor, leading global voice on sunlight research ✓
- "Cyndia" — signature on the automated internal notification emails ✓. Human or not: ?

2. Product and constraint (context only — not a workstream)

Phothera NB-UVB phototherapy devices. FDA-cleared **for dermatological conditions**. Every device requires a valid prescription from the patient's own healthcare provider ✓

- **Home line:** Phothera 200 · 200XL · 400 · 600 · 600-3D
- **Clinical line:** Pro 2400 · Pro HF · Pro 4000 XL · Pro 4800 Max ✓
- **Conditions routed in the live automation:** derm branch (psoriasis · vitiligo · eczema) · neuro/autoimmune branch (long Covid · MS) · anything else → Perplexity lookup ✓

Price: published nowhere ?. Category range roughly \$1K to several thousand. Every figure in this document is therefore **revenue at the \$2,000 floor unit, never profit.**

THE THREE CONSTRAINTS THAT SHAPE EVERYTHING

(a) Prescription-only — the funnel cannot end at a checkout. No patient can simply buy. Every sale requires a physician. Whatever is built must terminate in a signature, not a card payment ✓

(b) Channel conflict — their supplier is their competitor. Phothera is the former **Daavlin**; Daavlin + National Biological merged under **Madison Industries** ✓. phothera.com runs its own "Request Home Phototherapy" flow advertising "broad insurance coverage with \$0 cost for eligible patients", plus a dedicated dermatologist portal at photherahcp.com ✓. A two-person operation is reselling the devices of a Madison Industries company that owns its own DTC funnel, HCP channel and insurance

infrastructure. **Phothera runs no ads** and has just hired a digital marketing lead; its growth is rep-led into clinics ✓. → **CytoKind cannot win on derm generally. It can win on MS/autoimmune/long-Covid, which Phothera does not speak to.**

(c) The indication gap. NB-UVB for MS / long Covid / autoimmune is **research-stage, not an FDA-cleared indication** ✓. An MS patient gets no coverage and must pay \$2,000–\$7,000 cash. A derm patient is indicated, insurance pays, and the patient pays **\$0**. A pediatric MS indication is available — FDA already met CytoKind and said the data is sufficient; ~6 months, ~\$500K, requires a CytoKind-branded device 🚫. Out of scope here; it is the long-term unlock.

(d) They cannot supply their own prescriber. ✓ John, 2026-08-07:

"We can't even have our own doctors, because we need all the clinical notes for the insurance company. So we have to empower the patient to empower the doctor — or their staff — to get it done."

→ **Any telehealth-prescriber solution is dead.** The only viable mechanic is arming the patient to arm their own physician. **Patient education is therefore not a nicety — it is the revenue mechanism.** John calls the end state **"the touchless sale."**

3. Client acquisition — the true state

3.1 PAID — EVERYTHING IS FACEBOOK, AND IT IS LIVE ✓ (ADRIAN + AD-LIBRARY MEASUREMENT, 2026-08-14)

The account is no longer dark. Ads went live again on **11 August** after a period of approvals.

Active ads	4 — two creatives, each run as an A/B pair ✓
Creative 1	"Trusted by Dermatologists for 40+ Years" — image → /a-trusted-psoriasis-treatment-for-over-40-years/ ✓
Creative 2	"MS Fatigue. New Research." — video, podcast clip → /light-therapy-for-ms-fatigue-contact-us/ ✓
Landing pages behind ads	2 ✓
Arrivals	~20/day, company record ✓

🚫 **Half the live spend still points at MS** — no FDA indication, \$2–7K cash price, **zero devices sold there, ever**. The other half points at psoriasis, where insurance pays and **all six sales came from**. MS is kept running *"as a benchmark"* — that is a decision, not a system. 🚫 **Dropping MS is now being discussed internally** — *"whether we go dermatology only... and get rid of the MS for now."* Go in with a recommendation, not a question ✓

Earlier creative history: a video ad was flagged by Facebook for a **policy violation** and pulled; Craig reworked three skin-disease videos with *"prescription"* and *"disease"* removed ✓. Campaigns previously went dark on a **failed credit-card payment** ✓. **983 Group refused creative variety** when asked ✓ (*"I want a variety of ads running. And they were like, no."*)

The only disclosed numbers: | Metric | Value | |---|---| | Spend, psoriasis-related, to date | ~\$300 ✓ | | Cost per landing-page view | *"cents, less than a dollar"* ✓ | | Cost per form fill | **\$14** at best ✓ | | Total monthly ad spend | UNKNOWN ? | | Impressions · CTR · CPL · cost per booked call · cost per sale | UNKNOWN ? |

Benchmark for scale: Bioness/BNS Medical does ~\$30M/yr on MS foot drop with **~\$40,000/month** in Meta spend 🚫. CytoKind runs on a few hundred.

Other paid: Google Ads produced zero conversions and was terminated. The suspected cause — a high-friction form — was never validated ✓. If the cause was form friction, the same friction still exists in the Facebook path.

Measurement: no business goals are set in the platform — *"we don't even have business goals set up in the program"* ✓. **No Meta Pixel / Google Tag has ever been confirmed installed** on cytokind.net — assigned to John 2025-12-22, never verified ✓. The team has been **clicking its own ads**, polluting August's data ✓.

Reporting: 983 Group reports **bi-weekly** ✓. Tom, asked on 2026-08-07 to walk through what has and hasn't worked: *"I didn't come in prepared to, in full detail."* **The performance history lives in his head, not in a report.**

3.2 EARNED MEDIA — THE ONLY CHANNEL THAT HAS EVER PRODUCED REVENUE ✓

- **Scientific American cover, June 2025** — *"Surprising Ways That Sunlight Might Heal Autoimmune Diseases."* Embedded in every automated email. Drives daily traffic ⓘ (volume ⓘ).
- 🌟 **It closes roughly 1 in 3 of everyone who reads it.** The email sequence that carries it gets **1% CTR** on ~200 sends — so about **2 people in 200 ever reach it** ✓. **The highest-converting asset in the company sits behind its weakest delivery mechanism, and it never cost a media dollar.**
- **Conversion on earned media is ~10% when it should be 30%** — John, **6 Jan 2026**: *"probably 10%. It should be 30%, if I had a system about how to manage them when they came in."* Eight months later there is still no system ✓. **The oldest unfixed item in the file.**
- **Richard Weller** is being interviewed by **Huberman (~7M audience)**; has already done an NPR hour and *"talked for an hour and never mentioned us"* ✓. **No capture asset sits behind any appearance.**
- **The 66-page paper is nearly final**, out to UK co-authors ✓. A **cancer + heart-disease paper** (3–8% mortality signal, 300M-person dataset, Weller and Prue as co-authors) has no launch plan attached ✓. MS positioning = fatigue; cancer = *"survival and quality of life."*
- **The SciAm author's book tour** (Brooklyn → Vermont → Aspen, from June 2026) ran with no funnel attached ✓.
- **LITE trial** (home NB-UVB proven non-inferior to in-office) and the **MS publication** appear in no sequence, no page, no script ✓.
- **Aetna began covering home phototherapy for vitiligo in July 2026** — announced by Phothera. CytoKind is not marketing it ✓.
- **15 reels on Facebook/Instagram, all Adrian's, none performed** ✓ — *"doing social media management properly, we haven't done. And that's on me."* An open seat he names himself.
- Reddit MS communities · the "Before We Die" podcast · webinars · a social push to 1,000 followers — **all proposed 22 Dec 2025, none executed** ✓.

3.3 DOCTORS AND CLINICS — THE UNOPENED MULTIPLIER

- **~40–70 doctors have already written a CytoKind prescription and have never been contacted as a channel** ✓. John: *"instead of B to D — to the doctor who's then writing 100 scripts for us."* **The prescriber is the only actor whose single conversion yields many units. It is the only compounding channel in the business, and it is unbuilt.**
- **Blocked by a data failure:** the CRM tracks the patient, not the prescriber. John extracted NPI / doctor data **by hand, via Perplexity, off image scans** ✓. Flagged 6 Jan 2026, still open. **No prescriber exists as a record, so even repeat-script doctors are uncouneted, unreachable and unasked.**
- **Named as a priority three times across eight months. Never built** ✓.
- **Doctors approached so far respond with "how can I get a cut of each sale"** ✓ — the friction is economic, not scientific. **The Sunshine Act blocks commissions;** a compliant honorarium / thought-leadership structure was designed Dec 2025 and never built ✓.
- **Adrian is an internal advocate** — *"a doctor to patient model would be strong"* ✓.
- **A \$40,000 clinical unit was sold to a Buffalo centre in Dec 2025 and the motion was never repeated** ✓. One clinic ≈ 20 home units. A modelled alternative — install free, 50% rev-share year one / 25% year two — was valued at **\$390–400K** to CytoKind vs a 20% commission on a \$40K sale ⓘ.
- **The California group:** 10 offices, \$5–7M revenue, ~\$500K per office within 6–9 months, targeting 50 offices, nurse practitioners who can prescribe. Positioned at free-parking sites because patients attend 3x/week and dermatology complexes charge for parking. Identified Jan 2026, **never converted** ✓.
- **Medical College of Wisconsin** — 1,200 MS patients, high latitude. A free light in the practice ≈ **\$6,000** buys a registry, a publication and a referring centre. Never run ✓.
- **The head of lung cancer at Cedars-Sinai said** *"with the data you have right now, I would send you all of my patients."* **No follow-through** ✓.
- **CytoKind has no HCP-facing asset of any kind**, while Phothera runs a whole dermatologist site ✓.
- **LinkedIn Sales Navigator is owned and unused** — *"I don't optimize it"* ✓.

3.4 OWNED AUDIENCE AND PARTNERSHIPS

- ~50 patients own lights and are unmonetized — "we don't monetize that." Almost no patient emails are held. **No testimonials have been collected** ✓. Simultaneously the cheapest next sale and the missing proof asset for the doctor channel.
- **Function Health** (\$2.5B DTC blood-testing) — its users receive inflammation/MS-risk signals and have no action to take. CytoKind is the action. Never pursued ✓.
- **Blue Cross and Kaiser** both expressed interest in trials for late-stage MS patients to justify transitioning them off drugs. Never advanced ✓.
- **Israel/PTSD · Northern Europe (Edinburgh, Oslo, Copenhagen) · Middle East vitamin-D** — three geographies identified, none opened ✓.
- **Canada and Australia are writing in now** and being handed away or dropped ✓.

3.5 THE STRATEGIC SPLIT THAT WAS NEVER RESOLVED

DTC vs direct-to-clinic was agreed on 12 Jan 2026 to be decided — "focus on one primary lead gen approach for the next 6–12 months" ✓. **No choice was ever made. Both still run.** That window closes this quarter.

4. Funnel — the true state

4.1 THE LIVE PATH ✓

Facebook ad (4 live: 2 psoriasis image, 2 MS video)

- click
- LANDING PAGE (WordPress, cytokind.net — one of two)
- GRAVITY FORM [phone number now MANDATORY]
- SUBMIT
- ← back to Facebook. Nothing.
- ActiveCampaign holds the record
- Adrian calls / emails, manually, at 1 in 15–20

Plus the older organic path, still live: **4 WordPress forms** (appointment · product · contact · footer-email-only) → n8n → SPAM check → Google Sheet row → country routing → condition routing → AI-drafted reply **sent to John for approval, not to the lead** ✓. **Every non-US lead is given away** to an international distributor by that same automation ✓.

4.2 ★ THE SUBMIT BUTTON — WHERE THE FUNNEL ACTUALLY ENDS

This is the headline finding of the audit and the most recently proven one (14 Aug, live, while the ads were running).

"They fill out the form, which is a landing page, and then they hit submit... and that's the end of the experience." "The funnel ends after we get their contacts, but we should have an AI variable form that helps educate them." "So we're letting go of them while we have them." "They don't know what the JotForm is, they don't know that insurance covers it, they don't know that they need XYZ from their dermatologist."

At the single highest-intent second in the entire funnel — the moment after submit — nothing is delivered. No thank-you page. No education. No routing. No statement of cost. The patient is returned to Facebook.

WHERE IT LEAKS	NUMBER	WHY
The submit button	100% of arrivals	Returns to Facebook. Nothing delivered at peak intent
Price never stated	—	The one fact that closes (\$0 to the patient) is absent from the page they just filled in

WHERE IT LEAKS	NUMBER	WHY
Credibility	—	"It sounds like a scam" — verbatim, from a man who then bought
Phone	1 in 15–20	Cold outbound to someone with no context
Email	1 in 40	"I don't know if these emails are landing in spam"
Drip campaign	25% open · 1% CTR	4 emails. "It's not leading to anything"
JotForm	4 ever completed	It is the sale, and nothing drives anyone to it
Prescriptions parked	18–20	Decided demand, nobody owns the signature
Whole pipe	6 of 334 = 1.8%	

4.3 THE CAPTURE-ORDER FAILURE — THE SAME FINDING, ONE STEP UPSTREAM

Leads are captured **before** any education has happened. Adrian, 2026-08-07:

"I've sent countless emails, gone un-responded. I've talked with many patients on the phone and it's a matter of: what is this? Oh, I did that form one time. I have no context of what's going on. I'm barely invested and we're at the very beginning of the education cycle."

John reached the same conclusion independently, ten days apart:

"Why don't we educate them a little further before we ask for the information — so they're a little more invested." · "It's free information, it's not proprietary. Why don't we have that while they're still on Facebook?" · "We don't need the back end. The problem is way out top."

This is not a follow-up problem. It is a sequencing problem. And it is the single point where the 33%-closing SciAm article and the 1%-CTR email sequence intersect: **the asset that converts is on the wrong side of the ask.**

❗ **Form abandonment is invisible** — Tom: "if somebody doesn't fill out the form, we don't know anything about it." Cost is incurred at the click; the drop-off is unmeasured.

4.4 ★ THE FIX FOR THE BIGGEST LEAK WAS BUILT AND NEVER SWITCHED ON

Adrian built a **dynamic qualification form** with Claude. It asks where the psoriasis is, what insurance the patient carries, what symptoms they have — and ends with:

"Congratulations, your insurance will cover this light. Here's what you need to do. Here's the JotForm."
"And then, in theory, we don't need to talk to them. Ever."

Never tested. Never shipped. The CEO did not know it existed. John, on hearing about it: "Oh, that's a good surprise. I didn't know you were working on that." Adrian: "I built it and I don't know why we haven't put this."

This is the single strongest proof that the seat is empty. The fix for the biggest leak in the business sat in a folder, unknown to the CEO, for weeks — while ~20 people a day walked into the leak it was built to close. **It is an ownership gap, not a capability gap.**

On-call recommendation, cheapest test on the board: A/B the step immediately after the landing page — **funnel A = book a call · funnel B = straight to the JotForm.**

4.5 THREE FUNNEL SHAPES ALREADY TRIED AND FAILED ✅ — NEVER PROPOSE THESE AGAIN

ITERATION	JOHN'S OWN VERDICT
Meta ad → book a meeting directly	"most of the people weren't showing... it was an expensive lead, it was a no-show. It's wasting my time. It's wasting Adrian's."

Meta ad → contact list → follow-on emails "also been a challenge"

Meta ad → SciAm nurture → meeting "Did you increase the number of people attending? Not a whole lot."

The untried path is: the article first as the destination, the meeting earned afterwards.

4.6 THE JOTFORM — THE ONE UNAMBIGUOUS CONVERSION EVENT

Adrian built it with Claude. HIPAA-compliant. Captures the insurance card, the doctor's clinical notes, the NPI — "all the data that *Phothera needs*" to ship ✓.

"So if you fill out that form, it's a sale, basically." "We only have four. We've done it successfully four times. We do that a hundred times. We do it a thousand times. That's what we need."

Everything in the engagement should point at one number: **JotForms completed**. It is the only metric in this business that cannot be argued with.

4.7 DATA AND CRM

- ~138 people who reached out are in no CRM at all. John says ~400 have reached out; the CRM held 262 on 4 Aug ✓. The website form and a Google Sheet were never integrated.
- 2 of 3 ActiveCampaign automations do not function. Tags are decorative — nothing fires off them. "Next outreach date" is a free-text field, so no automation can read it ✓.
- 139 of 262 contacts are marked "originally imported" with unknown provenance ✓.
- Three CRMs in eight months: Google Sheet → Monday.com considered → ActiveCampaign ✓. History and behaviour lost each time.
- John has spent under 8 hours in ActiveCampaign and set its only goal himself ✓ — "its functionality isn't touched in any way, shape or form." He has since told Adrian to "take two days and learn the CRM" ✓. A CEO should never have to learn a CRM — this is the seat, restated.
- Prescriber data has never been captured — see §3.3.
- Patient health data currently sits in Google Sheets and Gmail !. No BAA is known to exist. Nobody owns this. ?

4.8 SPEED TO LEAD

There is no new-lead notification of any kind. No SMS, no push. Weeks pass ✓. John's own number: "100 minutes or 30 minutes versus 6 minutes is a 100x falloff."

4.9 NURTURE

- ~1,000 emails sent lifetime. Zero sales. 200 sends · 25% open · 1% CTR ✓. The open rate is fine; the click is the break, and the click is the article.
- No follow-up automation exists at all — every touch is manual, while John's own read is "a sale is four, five, six emails. You've got to touch on that often to close them" ✓.
- ! Evolv's email drafter produced zero usable output — "there wasn't a single email that got drafted that I didn't customize." Our bug. Own it unprompted.
- 983 Group's drip campaign was their first deliverable (June 2026); it now runs at 25% open / 1% CTR and "is not leading to anything" ✓.

4.10 THE PRESCRIPTION HAND-OFF — WHERE THE MONEY PARKS

- 18–20 prescription forms outstanding = \$36,000–40,000 at the \$2,000 floor unit ✓. These are patients who have already decided. Nobody is shepherding the doctor's signature.
- The patient must self-advocate to a physician who may never have prescribed home NB-UVB. No automated Letter of Medical Necessity exists ✓. A care-provider package was built with Craig in June 2026; usage never verified ?.

- **Insurance is the sale, and it lives only in John's mouth.** The automated email says "*Most insurers provide coverage for 'Medically Necessary' phototherapy... I can help provide a summary of the needed documentation.*" That message appears **nowhere on the website** — no insurance page, no HSA/FSA, no financing, no LMN template ✓. **Buyers therefore reach the phone not knowing the device is free to them.**
- **!** Their own blog post tells readers "*manufacturers have teams dedicated to helping you with the insurance process*" — pointing at Phothera, not at CytoKind ✓.

4.11 WHAT IS BEING BUILT RIGHT NOW (JOIN IT, DON'T DUPLICATE IT)

- **Tag architecture — Tom.** Right tags per funnel stage, redundant tags removed, plus **auto-tagging off the form's free-text box.** Document circulated ✓
- **SMS — Tom + Adrian.** Manual first to test messaging, then ActiveCampaign's paid SMS automation. Stated purpose: "*that frees you up, Adrian, to just focus on calls.*" ✓
- **The post-form education page — Adrian + Tom.** Built on **Netlify**; Adrian wrote the content, Tom did the UI/UX. Sits after the form: submit → within 5 minutes a text + email with everything the patient needs to take to their care provider. First draft, circulated, **not live** ✓
- **The dynamic qualification form — Adrian.** Built, never shipped. See §4.4 ✓
- **Craig's three reworked skin-disease videos** — made, emailed, not reviewed, not live ✓

5. Sales — the true state

5.1 COVERAGE

- **214 of 262 contacts (82%) had never been phoned** when the CRM was opened live on 1 Aug ✓. The 48 who *were* phoned produced **both** closed sales at that point. **The database has since grown to 334; calling capacity has not changed.**
- **A further ~100+ lukewarm leads sit uncalled** in Adrian's list ✓.
- **! The uncalled list is not the opportunity.** Calling every never-phoned contact at the observed connect and close rates is worth roughly **\$3,900** ✓. **The rate is the constraint, not the list.** Adding traffic does not add revenue while the constraint is downstream of the ads.
- **Sales capacity is one part-time person** — "*if Adrian catches a cold, then everything slows down*" ✓.
- Discovery calls run **1–2 per week**; roughly **1–2 meetings per 100 leads** ✓.
- 17 voicemails, 9 emails after a voicemail — **no systematic second, third or fourth touch after a miss**, while John's own read is that a sale takes 4–6 touches ✓.

5.2 PROCESS

- **Calls are not recorded.** The conversations that produced all six sales left no evidence, so nothing can be learned from the winning ones ✓.
- **The sales script was written by Craig, who has never made a sales call !.**
- **The connect rate is contradicted 4.6x:** tags say 48→22 (46%), Adrian says 1 in 15–20 (5–7%). **Neither figure is usable for planning** ✓.
- **No pipeline stages, no forecast. No-show rate UNKNOWN ?.**

5.3 SEGMENT FIT AT THE POINT OF SALE

The rep is being asked to sell a \$2,000–\$7,000 cash purchase to a ~65-year-old MS patient with no coverage ✓, while **all six sales that closed were covered derm cases where the patient paid \$0.** John: "*we're accidentally selling to them through our MS ads.*" Adrian: "*most of those are MS. The close rate out of DERM is pretty good.*" → **The mix is the cap, not the effort.** The fix is a routing and targeting decision, not "call harder."

5.4 REVENUE COLLECTION

- ★ ~50 units have been delivered to Phothera and \$0 has ever been invoiced. Adrian's estimate ~\$250K — **UNVERIFIED, never repeat as fact** ⚠. Money already earned, already delivered, sitting uncollected. No marketing, no leads, no ads required.
- Canadian leads arrive and are not served; the Phothera/Michael commission is not being taken ✓. Volume UNKNOWN ?.
- 983 Group's cost and the real monthly ad spend — **UNKNOWN** ?.

5.5 CURRENT OUTPUT

- 6 devices sold, lifetime ✓ (14 Aug). All six insurance-covered derm.
- Why 2 → 6 in ~1.5 weeks: the prescription process started about a month ago and is "starting to trickle in." **This is a lag effect, not a step change in demand** ✓.
- Adrian's goal reset: 20 sales between 10 Aug and 10 Sep. Currently at 6 ✓.
- Target 20/mo vs ~5 actual = a **\$30,000/month** gap at the \$2,000 floor unit ✓.
- 20 sales is also Alex's investment gate ✓ — the funnel is not just revenue, it is the raise.

5.6 OWNERSHIP AND CAPITAL

- ★ John, verbatim: "*no one is owning the whole pipe.*" ✓ Every individual link has an owner; the chain has none. **This is the reason every item above coexists.**
- The investor deck is **MS-heavy** — "*we need to move the numbers all the way to the front and the sides to the back*" ✓.
- Nobody is paid except 983 Group. John takes no salary and funds personally; Adrian \$2,500/mo; Craig unpaid ✓. Structurally fragile.

6. Competitive position

PLAYER	POSITION	RELEVANCE
Phothera (Daavlin + National Biological, Madison Industries)	CytoKind's own supplier. Owns derm end to end: DTC funnel, HCP portal, insurance infrastructure. Runs no ads; grows via reps into clinics; just hired a digital marketing lead ✓	Caps the derm strategy permanently. The answer to " <i>why buy here</i> " is not optional
Solius	Raised \$19M , FDA-cleared on a vitamin D claim, cites Richard Weller's research on its own site. ~6,800 monthly visits on the research page alone ✓	A funded competitor is monetising John's own science
Bioness / BNS Medical	~\$30M/yr on MS foot drop, ~\$40,000/month in Meta spend ⚠. Also a 983 Group / Tom account ✓	Proof the MS audience buys at scale — and Tom already runs a medical device account
Digital Silk	Running an audit of cytokind.net. Adrian has now given them CRM access; the audit lands next week ✓ (14 Aug)	★ A competing agency now holds the data and is executing our exact motion. Adrian: " <i>that could be Tom's ticket away from Cytokine.</i> " There is a clock on this
Inspire · Darien · Dexcom · Hims/Hers · Ro · Joovv · HigherDose · Bon Charge	Named by John as companies to study after the competitor-ad demo ✓	The demo that produced buying behaviour on the 2026-08-06 call

CytoKind's only true moat is MS — a Phase 2 trial published in *Multiple Sclerosis and Related Disorders* — which is precisely where there is no FDA indication ✓.

7. Voice of customer ✓

Verbatim inbound, pulled from the live automation's prompt corpus and the phones:

- "I have had MS and psoriasis for many years and now I have psoriatic arthritis as well / not sure which machine!" — two conditions, one device, no guidance
- "I got functional movement disorder with periodic paralysis, psoriasis, inflammatory arthritis, and debilitating brain fog post-Covid. My rheumatologist recommended phototherapy but insurance won't cover it so far. I'm losing hand function and mobility quickly. I can pay cash." — doctor-referred, ready to buy, still waiting
- "I listened to an article in Scientific American and was amazed by the results found in MS patients (in terms of fatigue)..." — the earned-media asset doing the entire job alone
- "My wife suffers from the autoimmune liver disease PBC... can any of your products help?" — a condition the site does not mention
- A German internist, post-Covid small-fiber polyneuropathy, wanting UVB **instead of Rituximab**
- "Can I get this product in Australia? / pricing needed" — demand from an unserved territory
- ★ "It sounds like a scam." — a buyer's first reaction on the phone. He checked the website himself, found the evidence, came back the next day and purchased. **He rescued himself.**

What it says: demand is urgent, emotional, cash-capable, and **overwhelmingly not standard dermatology**. Patients arrive from the SciAm article with autoimmune and neuro conditions asking whether the device applies to them. **The site is built for derm. The traffic is not.** That tension is the central commercial fact of the business — and it is a positioning decision, not a copy tweak.

8. Compliance — a hard constraint, not a footnote

- Devices are **prescription-only**; every claim is regulated ✓. The team already believes it cannot market normally because of the category.
 - **MS / long-Covid / autoimmune is not an FDA-cleared indication.** All such messaging must stay in evidence/research framing, never a treatment claim ✓. **Who signs off on copy: ? — nobody currently does.**
 - 🚫 **Facebook has already flagged a CytoKind video ad for a policy violation** (2026-08-07). Craig's fix was to remove the words "prescription" and "disease". **Creative compliance is a live, unowned risk on the only paid channel.**
 - **Patient health data sits in Google Sheets and Gmail.** No known BAA. Unowned 🚫 ?
-

9. The fourteen findings — the compressed layer

FINDING

- F01 The supplier is also the competitor.** Phothera runs a direct-to-patient flow, a dermatologist portal, and advertises \$0 insurance cost. The answer to "why buy here" is essential and currently unwritten
- F02 A funded competitor is already monetising the same science.** Solius raised \$19M on an FDA vitamin-D clearance citing CytoKind's own advisor; Bioness does ~\$30M/yr into MS on the same ad platform. The MS audience is proven to buy at scale
- F03 The traffic is autoimmune. The site is dermatology.** Inbound is overwhelmingly MS, long Covid, autoimmune fatigue and post-viral neuropathy — and MS is the only lane the supplier does not own *and* the only lane with no FDA indication
- F04 Earned media is the only channel that has ever produced revenue.** The SciAm article closes ~33% and never cost a media dollar. It is buried in a sequence that opens at 25% and clicks at 1%
- F05 Insurance is the sale — and it lives only in one person's mouth.** Indicated derm patients pay \$0. That fact appears in an automated email and nowhere on the website
-

- F06 The funnel ends at the submit button.** Ads work, page works, then the patient is returned to Facebook. One person then connects once per 15–20 attempts against ~20 arrivals a day
- F07 Leads are captured before they are educated.** The ask comes before the value. Not a follow-up problem — a capture-order problem. CEO and caller reached it independently, ten days apart
- F08 Half the paid spend targets a segment that has never sold.** Two of four live ads point at MS: no indication, \$2–7K cash, zero devices ever. All six sales came from the psoriasis half
- F09 One completed JotForm equals one sale. Four exist.** Meanwhile 18–20 prescriptions are parked — patients who already decided, waiting on a signature nobody chases
- F10 The fix for the largest leak was built and never switched on.** A dynamic qualification form that ends "*your insurance covers this — here's the JotForm*" sat in a folder, unknown to the CEO. Ownership gap, not capability gap
- F11 The list is already larger than the capacity to work it.** 214 of 262 never phoned; the base grew to 334; capacity unchanged. Calling everyone is worth ~\$3,900. Adding traffic does not add revenue
- F12 The prescriber channel is the unopened multiplier.** The only actor whose single conversion yields many units. Named three times in eight months, never built, and no prescriber exists as a record
- F13 Eleven of twelve numbers do not exist.** Form abandonment invisible; calls unrecorded; two of three automations dead; tags decorative; under 8 hours ever spent in the CRM; two connect rates that disagree 4.6×
- F14 Nobody owns the whole pipe.** Every seat is filled and every seat is doing its job — which is exactly why F06, F09 and F10 are all true at once. The gap is not a missing person; it is that no function owns the distance from impression to invoice

10. Corrections — earlier beliefs now overturned

EARLIER BELIEF	WHAT IS ACTUALLY TRUE	SOURCE
No landing page — Meta instant form only	A real WordPress landing page with a Gravity Form feeding the CRM exists	07 Aug
The phone field drives people away	The drop-off came from the CTA copy change (" <i>Contact us</i> " → " <i>Set up a consultation</i> "), not the phone field. Phone is now mandatory	07 Aug
Nothing is running — zero ads, zero landing pages	Four ads live since 11 Aug behind two landing pages; ~20 new contacts a day, a company record	14 Aug
Tom makes the creatives	Craig makes the creatives. Tom writes captions, sets up the ads and vibe-coded the landing page	14 Aug
Daavlin is a competitive benchmark	Daavlin is Phothera, the supplier. The frame is channel conflict, not competitive catch-up	28 Jul
A telehealth prescriber would unlock the prescription step	Structurally impossible — insurance requires the treating physician's own clinical notes	07 Aug
Effort is the constraint — more calls, more emails	Mix is the constraint. Derm converts; MS never has	14 Aug
The uncalled list is the opportunity	Calling every never-phoned contact at observed rates is worth ~\$3,900. The rate is the constraint, not the list	04–11 Aug
14 prescriptions parked / 262 contacts / 2 sales	18–20 parked · 334 contacts · 6 sales · 1.8% close	14 Aug

Two Evolv-side errors, recorded in the same spirit: our automated email stripped the phone number from the lead notification (John concluded phones were not being captured; they were), and our early email drafter produced no usable output.

11. What is still open — and what closes each one

OPEN ITEM	WHAT IT IS	WHAT CLOSSES IT
Ad performance history	Total monthly spend, MS vs derm split, impressions, CTR, cost per booked call, cost per sale. Held by 983 Group; the scheduled review arrived without detail	One report
Form abandonment	How many reach the form and leave. Never instrumented — cost is paid at the click, the loss is invisible	One tag
Delivered vs invoiced	~50 units shipped, reconciled against units invoiced. Never performed. Material but unverified	One afternoon and a spreadsheet
People not in the CRM	Enquiries believed to exceed contact records by ~138. Website form and intake sheet never integrated	One import
Medical copy sign-off	Every MS/autoimmune claim is regulated and no named person approves wording	One decision
Where health data sits	Conditions, history and prescriptions moving through spreadsheets and inboxes. BAA coverage and visibility never assessed	One assessment
Distribution agreement scope	What Phothera actually grants — territory, exclusivity, margin	Ask John
Cash-pay vs insurance-billed mix	Do they bill insurance at all, or only advise?	Ask John
Canadian lead volume	And the Phothera/Michael commission not being taken	Ask Adrian
Clinical-line motion	Whether a sales motion for the Pro line exists at all	Ask John
Craig's Mega AI persona profile	Committed to in advance, never sent	Ask Craig

Two questions deliberately never asked, and never to be asked again: margin per unit and average selling price. Pricing everything on the \$2,000 floor unit makes every figure a minimum and removes the need for either.

12. Sources — fourteen, across eight months

CODE	CALL	DATE	RECORDING	KEY OUTPUT
D0	Evolv × CytoKind — 7 lead-gen strategies	2025-12-22	110389814	Original channel map; the clinic sale never systemised
D1	UPDATE EVOLV AI (John, full strategy)	2026-01-06	112025067	"It should be 30%, if I had a system"
D2	Impromptu — DTC vs direct-to-clinic	2026-01-12	113479494	Agreed to pick one; both still run
—	John + Alexander + Adrian	2026-04-28	141730772	Investor thread
—	Follow-up	2026-05-04	143260058	—

CODE	CALL	DATE	RECORDING	KEY OUTPUT
E1	John + Adrian + Whiz — B2B2D / Alex / Craig	2026-06-03	151978270	"the doctor who's writing 100 scripts for us"; 4–6 touches per sale
J1	John update	2026-07-27	167585630	Speed: "a 100× falloff"
S1	CytoKind sales strategy	2026-07-27	167610255	The three abandoned funnel shapes
A1	Adrian update	2026-07-28	168068285	Pipeline from the phone operator
WEB	cytokind.net audit	2026-07-28	live site	No price, no checkout, no insurance page; MS buried
A2	Adrian funnel walkthrough	2026-08-01	169359140	214 of 262 never phoned
CRM	Live ActiveCampaign export	2026-08-04	live system	First baseline
J2	John — competitor demo, price asked	2026-08-06	170788812	"Don't say what's wrong. Say where you're taking us."
T1	Tom + Adrian + John + Craig + Whiz — ads working session	2026-08-07 !	transcript only	"The problem is way out top."
ADS	Meta ad-library measurement	2026-08-14	live account	4 active ads, 2 creatives, 2 landing pages
A3	Adrian, ~44 min	2026-08-14	173377987	★ The break point. Every number rewritten

Documents: 00-CYTOKIND.md · \$0 · 03-COMPETITORS.md · 03-COMPETITOR-FUNNELS.html · \$07 · 04-CALL-john-2026-08-06.md · 05-CALL-tom-adrian-2026-08-07.md · 08-CALL-adrian-2026-08-14.md · problems/00-FUNNEL-BLEED-2026-08-04.md · problems/03-BOTTLENECK-INVENTORY.md · research/00-website-audit-2026-07-28.md · pages/01-DISCOVERY.html · live audits of cytokind.net, phothera.com, photherahcp.com

13. Evolv's own record — own these before presenting anything

! Our auto-email stripped the phone number	John concluded they weren't capturing phones at all. Adrian had them the whole time. Own it unprompted.
! Our email drafter produced 0 usable output	"there wasn't a single email that got drafted that I didn't customize." Own it unprompted.
Commercial reality	Evolv charges ~€400/mo to triage the CEO's email at a company whose entire revenue engine is unowned, with Adrian already embedded inside
! The commercial shape is now overdue	Two people have asked in writing. John, 6 Aug: "what's the financial proposal?" Adrian, twice on 14 Aug: "Are you proposing to do some of the work... what level of involvement?" The ambiguity is a liability now, not a soft-sell.
Original scope, never closed	4 macro AI systems at €3,800 upfront + €1,800/mo were presented; ~€400/mo was bought. Why the rest never closed is recorded nowhere ?

14. Handling rules — non-negotiable

- ★ **Never repeat John's off-the-record remarks about replacing the ad team.** He asked explicitly.

- **Do not attack 983 Group.** Nigel owns the relationship, and their PR side genuinely works. Say so out loud — it keeps the critique credible. **Adrian is not an ally in removing Tom** (*"I believe Tom is doing a great job bringing in leads, but the leads are not educated enough"*) — he is an ally in fixing what happens after the click.
 - **Never quote profit.** Margin per unit is UNKNOWN and will stay that way. Say device volume.
 - **Never ask about margin per unit or ASP again.** Everything is priced on the \$2,000 floor unit.
 - **The ~\$250K receivable is Adrian's guess**, not a fact.
 - **Own our two bugs** (stripped phone, email drafter) unprompted before presenting anything.
 - **Do not re-propose the three failed funnel shapes** (\$4.5).
 - **Telehealth prescriber is impossible** — do not propose it (\$2d).
 - **★ John's coaching, obey it literally:** *"Don't say what's wrong. Say where you're going to take us. It's not a blame game."* → **This audit is internal fuel. None of it appears on screen in front of the team.**
-

"No one is owning the whole pipe." — John MacMahon

The one line this audit exists to prove. It determines everything in stages 02–06.

Numbers

Every figure below is taken from a recorded call or a live system, with its date. Nothing is estimated, benchmarked or forecast. All money is calculated on the \$2,000 Phothera 200, the cheapest device CytoKind sells — so every dollar figure is a floor, not a claim.

● **WORKING** — an asset today

● **PARKED** — awaiting a decision

● **UNMEASURED** — no number exists

● **LEAKING** — money lost now

1_{/12}

Business metrics that exist

Eleven of the twelve numbers this company runs on are not measured at all. It is the finding the audit repeats most often.

100%

Of arrivals returned to Facebook

The moment after submit is the highest-intent second in the funnel, and nothing is delivered into it. No page, no education, no price.

1 in 20

Connect rate on the phone — everything after submit

One part-time person, connecting once per fifteen to twenty attempts, against twenty arrivals a day.

4

JotForms ever completed

The one unambiguous conversion event in the business — “if you fill out that form, it’s a sale” — completed four times in the company’s history.

18–20

Prescriptions parked

People who have already decided, waiting on a signature that nobody currently owns. \$36,000–40,000 at the floor unit.

50%

Of paid spend, aimed at a segment that has never sold

Two of the four live ads point at MS: no FDA indication, \$2–7K in cash, zero devices ever. All six sales came from the other half.

~20_{/day}

New contacts arriving now

The highest volume the company has ever had — walking into the funnel described above.

\$30,000_{/mo}

The gap, valued at the floor

Between devices sold in the last thirty days and the stated target for the same period, priced on the cheapest unit in the catalogue.

“No one is owning the whole pipe.”

John MacMahon — the one line this audit exists to prove, and the one that determines everything in stages 02 through 05.

Sources: recorded calls of 22 Dec 2025, 06 Jan, 12 Jan, 03 Jun, 27 Jul (x2), 28 Jul, 01 Aug, 06 Aug, 07 Aug and 14 Aug 2026; the live ActiveCampaign export of 04 Aug 2026; the Meta ad-library measurement of 14 Aug 2026; and the cytokind.net audit of 28 Jul 2026. Where a figure has changed since it was first recorded, the 14 Aug value is the one shown.